PEER REVIEW HISTORY

BMJ Open publishes all reviews undertaken for accepted manuscripts. Reviewers are asked to complete a checklist review form (**http://bmjopen.bmj.com/site/about/resources/checklist.pdf**) and are provided with free text boxes to elaborate on their assessment. These free text comments are reproduced below.

ARTICLE DETAILS

TITLE (PROVISIONAL)	Understanding the beliefs and attitudes towards mental health problems held by Muslim communities and acceptability of Cognitive Behavioural Therapy as a treatment: Systematic Review and Thematic Synthesis Protocol
AUTHORS	Alharbi, Hind; Farrand, Paul; Laidlaw, Ken

VERSION 1 – REVIEW

REVIEWER	Naeem, Farooq University of Toronto Faculty of Medicine, Psychiatry Have published in this area, but no links with the authors
REVIEW RETURNED	23-Jan-2021

GENERAL COMMENTS	This is an interesting topic. However, the paper needs significant revision. Please consider the following; The title is a little confusing “understanding the beliefs and attitudes towards mental health problems held by Muslim communities and acceptability of Cognitive Behavioural Therapy as a treatment”. The title uses general terms, i.e., beliefs and attitudes towards mental health problems. However, throughout the paper, authors use the terms beliefs towards causes and treatment of mental illness. These discrepancies in terminology need to be addressed throughout the paper (beliefs about treatment, differences in beliefs across the countries). Please review the use of terms in title, abstract, objectives etc. How will authors compare beliefs across the countries? As far as I know, there are only 3 or 3 qualitative papers on the acceaptability of CBT from outside the west, two of which were published from India. There are many feasibility and acceptability studies, though (published as part of smaller pilots mostly). I suggest authors focus on the acceptability of evidence-based or just psychosocial interventions among Muslims in the title, as the readers might find this attractive. Many Muslims live in Europe and North America. Consider other European languages, eg France has the largest Muslim population (you will find many articles with English abstract only, so that needs to be considered). I am also not sure why beliefs about mental health and its causes and treatment in general and about CBT are being covered in one review. Please give a rationale for this. Page 6, Line 52- CBT, especially culturally adapted CBT is acceptable in many Muslim countries, including Saudia. Please give a reference or remove the line. There are more than 30 RCTs of CBT and other evidence-based psychological interventions (mostly adapted) published from Muslim countries.

	It is usually a combination of religious and cultural factors that influences our beliefs. At least mentioning this in the background might help the readers. The barriers to access might be different across countries. This can go into background discussion, which needs to be improved. Page 7, line 29- background ends abruptly. You can use this to prepare readers for what is coming next! The aims and objectives section is confusing and needs to be revised. Page 7, line 54- what are different Muslim communities? Do you mean Muslims living in different parts of the world? Compare beliefs of Arabs in the UK with Arabs in the middle east? Or Muslims in Kuwait with Muslims in Indonesia? Study Population: The study population includes working-age male and female adult Muslims that have not received psychological therapies for the treatment of mental health difficulties as well as patients with experience of CBT treatment. So those who have not received PSIs but those who have received CBT? Why will you exclude those who have received PSIs? Interventions- However, studies focused on acceptability towards psychoeducation and treatment approaches will be restricted to High and Low-intensity CBT. Why? High and low-intensity CBT is not a term used internationally. Search strategy- while the search terms in the appendix seem comprehensive, this section can be further improved with the help of a librarian The analysis process is described well

REVIEWER	Thomas, Justin Zayed University, Psychology
REVIEW RETURNED	08-Feb-2021

GENERAL COMMENTS	I think the aims of the project are worthwhile, although poorly framed. I find sections of the introduction objectionable, bordering on negative stereotyping. I think this needs a re-write with much more open mind and greater religio-cultural sensitivity Specific points The background to the abstract seems a little vague. “Many beliefs regarding mental health difficulties alongside attitudes toward psychotherapy and Cognitive-Behaviour Therapy (CBT) in particular are held by people within Muslim communities.” We might unpack the term “muslim communities”. Are we discussing sects (sunni / shia) madthhabs (Hanafi/hambali) or geography, language groups or a mixture of all. A statement of what we mean by Muslim communities is needed. By omitting beliefs in medical explanations too, the work suggests that metaphysicl explanations are dominant among these communities. I would suggest that is highly debatable within some groups.

	"There are many beliefs held among Muslim communities concerning the aetiology of mental health difficulties, including Evil Eye, possession and black magic". Much of this is very contestable and this following statement for sure needs a citation. "Such beliefs potentially result in psychological therapies, such as Cognitive Behavioural Therapy (CBT), being rejected by many in Muslim communities with a preference for traditional treatment" Terms like "many" are very vague, can we talk %. The above statement is not in my experience true at all. I would suggest a minority of Muslims would reject CBT on the basis of beliefs that all mental illness is caused by whispering "devils". Not at all sure why the authors then lump BAME in with people who wont seek CBT because of "devils"…." with specific barriers identified for members of the Black Asian and Minority Ethnic (BAME) community13." Overall I think the introduction and aim to this study are not written with enough insight or religio-cultural sensitivity. It seems to apply old stereotypes to "muslims" some of whom work in mental health a psychotherapists. I think the introduction needs to be rewritten with greater specificity and a broader view of diversity of opinion among the "muslim communities"

VERSION 1 – AUTHOR RESPONSE

Reviewer comment	The Reply	Modification (all changes highlighted in yellow on the revised manuscript)
Reviewer: 1. Dr. Farooq Naeem, University of Toronto Faculty of Medicine		
The title is a little confusing "understanding the beliefs and attitudes towards mental health problems held by Muslim communities and acceptability of Cognitive Behavioural Therapy as a treatment". The title uses general terms, i.e., beliefs and attitudes towards mental health problems. However, throughout the paper, authors use the terms beliefs towards causes and treatment of mental illness. These discrepancies in terminology need to be addressed throughout the paper (beliefs about treatment, differences in beliefs across the	* Thank you so much for this comment, this made us rearrange our thoughts and clarify some points better. Whilst the title was approved by PROSPERO, we have removed the focus on CBT from the stated aim for the paper and restated it as an objective to reduce the emphasis in the paper.	* Several points in the introduction have been changed to be clearer. Page 4,5 (Paragraph 1,2,3 in introduction). * The aim and objectives has been rewritten to be clearer and more specific. Page 6. Aim Understand barriers that prevent adult Muslims from accessing mental health services with comparisons undertaken between Muslim communities resident in different countries. Objectives: Gain an understanding of

countries). Please review the use of terms in title, abstract, objectives etc.		barriers that prevent Muslim communities resident in different countries accessing mental health services. Appreciate beliefs and attitudes regarding causes and treatment of mental health difficulties. Evaluate the extent to which beliefs held towards mental health and treatment vary between Muslim communities resident in different countries. Appreciate the extent that CBT may address barriers for adult Muslims.
How will authors compare beliefs across the countries?	During thematic synthesis any consistent differences that emerge between Muslims resident in different countries will be highlighted, specifically reported and discussed.	We have explained it on the page 10 (Following thematic synthesis, results will be compared across studies to address review objectives and reach an understanding regarding the barriers to access and acceptability of CBT to address these barriers. Where they arise, comparisons will be made regarding these barriers with respect to country of residence.)
As far as I know, there are only 3 or 3 qualitative papers on the acceptability of CBT from outside the west, two of which were published from India. There are many feasibility and acceptability studies, though (published as part of smaller pilots mostly). I suggest authors focus on the acceptability of evidence-based or just psychosocial interventions among Muslims in the title, as the readers might find this attractive.	I agree with the reviewer that focusing on the acceptability of evidence-based or just psychosocial interventions will be more attractive. Thank you so much for this comment, this made us rearrange our thoughts and clarify some points better.	As discussed above parts of the introduction have been rewritten to reduce the previous emphasis placed on CBT.
Many Muslims live in Europe and North America. Consider other European languages, eg France has the largest Muslim population (you will find many articles with English abstract only, so that needs to be considered).	Thank you for this suggestion, there is also a large number of studies published in Turkish, Farsi, and French. However, given resource limitations only papers published in English and Arabic language will be included. This is mentioned as one of the limitations of this review.	(Studies restricted to those published in English or Arabic since 1980.) Page 3
I am also not sure why beliefs about mental health and its causes and treatment in	The main focus of the review is to appreciate barriers that prevent different Muslim communities	CBT was identified as one of the objective only

general and about CBT are being covered in one review. Please give a rationale for this.	accessing mental health services. Previous research that has not focussed on the Muslim community has identified individual level barriers related to beliefs and attitudes towards the psychological therapy as a barrier. Given a focus of this review is to examine differences in barriers between Muslim communities resident in different countries and CBT representing the significant psychological therapy provided (the main therapy provided in the IAPT programme in England) it was decided to include this focus also. This was approved by the review informing Prospero registration. However, we do acknowledge the reviewers comment here and have reduced the emphasis In this paper on CBT and this area now represents a secondary objective. We hope reducing the emphasis on CBT within this paper is found acceptable.	(Appreciate the extent that CBT may address barriers for adult Muslims.)
Page 6, Line 52- CBT, especially culturally adapted CBT is acceptable in many Muslim countries, including Saudi. Please give a reference or remove the line. There are more than 30 RCTs of CBT and other evidence-based psychological interventions (mostly adapted) published from Muslim countries.	Thank you for this observation, the use of the word (reject) was exaggerated. However, as indicated above, whilst there may be increased acceptability towards culturally adapted CBT, little remains known regarding remaining barriers in Muslims residing in different countries and how such barriers may influence mental health treatment seeking. Therefore, the first option for patients (as mentioned in Dr. Naeem book cited in the manuscript) remains the traditional treatment based on religious therapy. Understanding the range of barriers may help infirm wider aspects of treatment seeking for CBT if there is improved acceptability.	Sentence has been modified, and add a new reference, page 5,6 (Whilst many Muslims believe that mental health difficulties represent medical problems, some believe the aetiology of mental health difficulties is associated with beliefs in Evil Eye, possession, black magic9 or as a punishment from Allah for sinful acts. Additionally, limited access to mental health services for Muslims has also been reported, resulting in treatment sought from religious healers rather than mental health services. Whilst some barriers to access are known, little is known about how these differ across Muslim communities resident in different countries.)
It is usually a combination of religious and cultural factors that influences our beliefs. At least mentioning this in the background might help the readers. The barriers to	Many thanks for pointing this out. The difference between countries has now been mentioned.	This has been added to the background, page 4. (Muslims may be affected and differ by their residential and cultures norms. For example,

access might be different across countries. This can go into background discussion, which needs to be improved.		although the Qur'an clearly states that certain behaviours such as drinking alcohol and eating pork are forbidden, it has been reported that 25% of Korean Muslims drink alcohol with 16% eating pork. Participants in this study reported that engaging in such behaviours arose as a response to their cultural upbringing, indicating that differences exist between Muslim communities residing in different countries.
Page 7, line 29- background ends abruptly. You can use this to prepare readers for what is coming next!	Thanks for the comment.	I have added a sentence to describe the review, Page 5. (This systematic review seeks to gain a fuller understanding concerning barriers that prevent Muslims in different countries from accessing mental health service with a secondary focus examining barriers that prevent Muslims from accessing CBT as treatment. Examining barriers effecting Muslim communities resident in different countries will improve understanding regarding the extent to which any differences permit conclusions to be generalised across community)
The aims and objectives section is confusing and needs to be revised.	Thanks for the comment.	* The aim and objectives has been rewritten to be clearer and more specific. Page 6 Aim Understand barriers that prevent adult Muslims from accessing mental health services with comparisons undertaken between Muslim communities resident in different countries. Objectives: Gain an understanding of barriers that prevent Muslim communities resident in different countries accessing mental health services. Appreciate beliefs and attitudes regarding causes and treatment of mental health difficulties. Evaluate the extent to which beliefs held towards mental health and treatment vary between Muslim communities

		resident in different countries. Appreciate the extent that CBT may address barriers for adult Muslims.
Page 7, line 54- what are different Muslim communities? Do you mean Muslims living in different parts of the world? Compare beliefs of Arabs in the UK with Arabs in the middle east? Or Muslims in Kuwait with Muslims in Indonesia?	Thanks for this comment, I agree with the reviewer, the use of the term (Muslim communities) was ambiguous. Therefore, we have identified Muslims according to their country of residence, to include Muslims who live in Muslim countries and Muslims who live as minorities in non-Muslim countries.	All references to Muslim communities now states 'resident in different countries' to enhance clarity.
Study Population: The study population includes working-age male and female adult Muslims that have not received psychological therapies for the treatment of mental health difficulties as well as Patients with experience of CBT treatment. So those who have not received PSIs but those who have received CBT? Why will you exclude those who have received PSIs?	Please see the comment above. That CBT represents the predominant evidence based psychological therapy provided in non-Arab countries, combined with resource limitations to undertake the systematic review it was decided to focus solely on CBT. However, we acknowledge this may serve as a limitation so have stated this in the Strengths and Limitation section.	(Due to limited resources, in this review focus has only been on CBT) Page 3
Interventions- However, studies focused on acceptability towards psychoeducation and treatment approaches will be restricted to High and Low-intensity CBT. Why? High and low-intensity CBT is not a term used internationally.	Thanks for the comment. Only cognitive behavioral therapy (CBT) is now referred to. However it should be noted that the distinction between low and high intensity CBT is increasingly being recognised in many countries and informing service delivery.	
Search strategy- while the search terms in the appendix seem comprehensive, this section can be further improved with the help of a librarian	Thanks for pointing this out. We are happy the reviewer found the search terms were comprehensive. A librarian was involved in developing and refining these search terms and we have now indicated this in the text as we recognise it is a strength.	This is indicated in the description of the strategy, Page 10. (HaH reviewed the research strategy with the Exeter University Librarian)
The analysis process is described well	Thank you.	
Reviewer: 2. Prof. Justin Thomas, Zayed University		
I think the aims of the project are worthwhile, although poorly framed. I find sections	Thanks for this comment, I agree with the Reviewer that there were some unclear terms and these	* Several points in the introduction have been changed to be clearer. Page 4,5

of the introduction objectionable, bordering on negative stereotyping. I think this needs a re-write with much more open mind and greater religio-cultural sensitivity	have been addressed. That there are differences between Muslim communities resident in different countries has now been made clearer with increased attention directed towards supporting all claims made and demonstrated by previous studies and this has been addressed through references. This observation has improved the paper by placing greater emphasis on differences in barriers that may exist between Muslim communities.	(Paragraph 1,2,3 in introduction). * The aim and objectives has been rewritten to be clearer and more specific. Page 6. Aim Understand barriers that prevent adult Muslims from accessing mental health services with comparisons undertaken between Muslim communities resident in different countries. Objectives: Gain an understanding of barriers that prevent Muslim communities resident in different countries accessing mental health services. Appreciate beliefs and attitudes regarding causes and treatment of mental health difficulties. Evaluate the extent to which beliefs held towards mental health and treatment vary between Muslim communities resident in different countries. Appreciate the extent that CBT may address barriers for adult Muslims.
The background to the abstract seems a little vague. "Many beliefs regarding mental health difficulties alongside attitudes toward psychotherapy and Cognitive-Behaviour Therapy (CBT) in particular are held by people Within Muslim communities."	Many thanks for pointing this out, I hope you find the revision clearer.	The background to the abstract has been rewritten to be clearer and more specific. Page 2 (Many barriers prevent Muslims' accessing mental health services, The aim of this systematic review is to gain an understanding regarding these barriers and consider how they vary across different Muslim communities resident in different countries.)
We might unpack the term "Muslim communities". Are we discussing sects (sunni / shia) madthhabs (Hanafi/hambali) or geography, language groups or a mixture of all. A statement of what we mean by Muslim communities is needed.	Thank you for this comment and we fully acknowledge the confusion this caused. We have now only used the term Muslim community when referring to Muslim communities "resident in different countries" and not made reference to 'sects'	All references to Muslim communities now states 'resident in different countries' to enhance clarity.

By omitting beliefs in medical explanations too, the work suggests that metaphysical explanations are dominant among these communities. I would suggest that is highly debatable within some groups. "There are many beliefs held among Muslim communities concerning the aetiology of mental health difficulties, including Evil Eye, possession and black magic". Much of this is very contestable and this following statement for sure needs a citation.	Many thanks for pointing this out. We have more clearly stated that there is a good belief in medical explanations to increase emphasis, alongside stating that belief in metaphysical explanations are also evident. Such claims are fully supported by reference to the literature	This sentence has been rewritten to be clearer. Page 4 (Whilst many Muslims believe that mental health difficulties represent medical problems, some believe the aetiology of mental health difficulties is associated with beliefs in Evil Eye, possession, black magic, or as a punishment from Allah for sinful acts)
"Such beliefs potentially result in psychological therapies, such as Cognitive Behavioural Therapy (CBT), being rejected by many in Muslim communities with a preference for traditional treatment" Terms like "many" are very vague, can we talk %. The above statement is not in my experience true at all. I would suggest a minority of Muslims would reject CBT on the basis of beliefs that all mental illness is caused by whispering "devils".	Use of the 'many' has now been dropped, I hope you find it more acceptable.	Sentence has been modified, and add a new reference, page 4,5
Not at all sure why the authors then lump BAME in with people who wont seek CBT because of "devils"…." with specific barriers identified for members of the Black, Asian and Minority Ethnic (BAME) community13."	I apologise, this is obviously an oversight and I have now addressed	Modified
Overall I think the introduction and aim to this study are not written with enough insight or religio-cultural sensitivity. It seems to apply old stereotypes to "muslims"	I apologise the reviewer felt some aspects of this paper lacked 'cultural sensitivity', especially regarding differences between Muslim communities. Extensive revisions have been made	The differences between Muslim communities are clarified, the background rewritten, and the discussion and objectives have hopefully been made clearer.

some of whom work in mental health a psychotherapists. I think the introduction needs to be rewritten with greater specificity and a broader view of diversity of opinion among the “muslim communities”	throughout to highlight differences that exist between communities resident in different countries. I hope the reviewer finds the revisions more acceptable.	